



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
980 9th Street, Suite 500
Sacramento, CA 95814
Phone: 916-324-8176 | Fax: 916-255-5241
www.DMHC.ca.gov

April 8, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Santa Clara Valley Medical Center
Block Transfer Filing No. 2026-HOSP-281
Filed on March 18, 2026

Dear Ms. Karen Choate:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Santa Clara Valley Medical Center** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. On the Block Transfer Filing form, when asked, "Is the Terminating Provider Group part of a "narrow network" product offered by the Plan?", the Plan answered, "yes".

Regarding the Narrow Network, **Pathway EPO Exchange Network**, please explain or confirm:

- a. The number of Enrollees in the Narrow Network Product,

- b. Any inpatient hospital-based services available to affected enrollees at Terminating Hospital that would not be available at any of the Alternate Hospitals,
- c. For any hospital-based services no longer available to affected enrollees post termination, how enrollees will continue to access such services. If any, please confirm that the enrollees will be able to receive these services at in-network benefit levels,
- d. Any admitting privilege issues to the Alternate Hospital(s),
- e. Name of Provider Groups that Admit to the Terminating Hospital as Part of the Product, and
- f. The number of admits and bed days utilized by enrollees for the most recently completed calendar year, by enrollees in the Plan's reported Narrow Network.

Plan Response1: Please refer to the responses below, corresponding to items a–f above.

- a. There are 7,179 EPO Exchange members residing within 15 mile radius of the terminating hospital.
- b. Some tertiary care would not be available (Burn services, as the alternatives are not designated as burn centers, although they do offer some level of burn services; and, PICU). Pls refer to c.
- c. The Plan does not anticipate any problems with the ability of the alternate hospitals to provide the same out-patient and/or in-patient services available at the terminating facility, with the exception of some tertiary care services (pls refer to b). If there is a service identified that is a covered benefit and only available at the terminating hospital the Plan will make every effort to ensure services are provided at the terminating hospital. When a covered service is not available or accessible at a participating network hospital or specialty facility in the Plan's' network members will be allowed to receive that service at a non-participating hospital as appropriate; reimbursement at in-network benefit level.
- d. Based on the information currently available to the Plan, no admitting-privilege issues have been identified for the Alternate Hospital(s)
- e. None. EPO members can utilize any PCP within the EPO Exchange Network.
- f. There were 7 EPO admits and 59 bed days (Note: This EPO Utilization is included in the PPO-reported totals (48 admits/448 bed days).

Please submit the Plan's response by **April 15, 2026**, via the Block Transfer portal as a "Response to Additional Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in blue ink that reads "Cindy Nguyen". The signature is written in a cursive style and is placed on a light yellow rectangular background.

Cindy Nguyen
Analyst II
Office of Plan Monitoring